



Stroke Patient Escalation Protocol

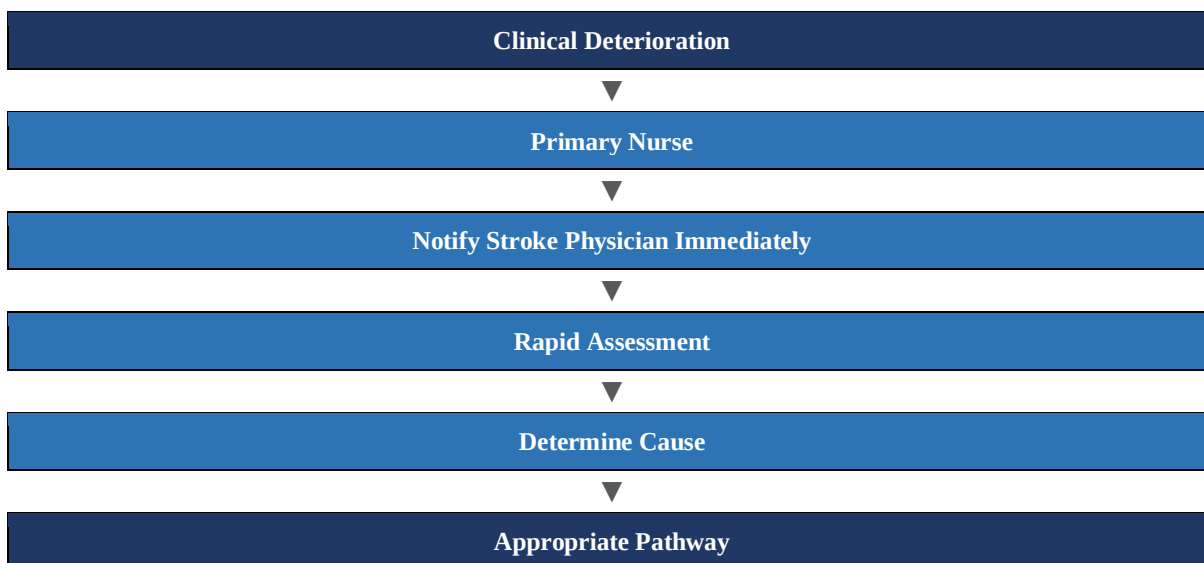
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Stroke Program — Clinical Escalation

Purpose

To provide a standardized response when a stroke patient clinically deteriorates.

General Escalation Flow



Deterioration-Specific Pathways

Neurological Deterioration

Examples: NIHSS increase ≥ 4 · GCS decline · new weakness · new aphasia

- Immediate physician review
- STAT CT Brain
- Consider CTA
- Notify neurosurgery
- ICU transfer if needed

Hemorrhagic Transformation

- Stop thrombolytic/anticoagulant if applicable
- Repeat CT
- Reverse anticoagulation if indicated
- Neurosurgery consult
- ICU admission

Airway Compromise

- Respiratory therapy
- Anesthesia
- Intubation
- ICU

Seizures

- Notify stroke physician
- Lorazepam
- Antiepileptic loading
- EEG
- Neurology review

Blood Pressure Crisis

- Follow IV antihypertensive protocol
- Frequent monitoring
- Escalate to ICU if uncontrolled

Cerebral Edema

- Repeat CT
- Hyperosmolar therapy
- Neurosurgery consult
- ICU

Cardiac Complications

- ECG
- Troponin
- Cardiology consult
- Telemetry

Documentation

All escalations must be documented in:

- Progress notes
- Stroke flowsheet
- Escalation record
- ICU transfer note (if applicable)

AHA/ASA Guideline Alignment & Accreditation

Guideline concordance: This escalation protocol operationalizes the neurological monitoring, hemorrhagic transformation, and blood pressure management principles of the 2026 AHA/ASA Guideline for the Early Management of Patients with Acute Ischemic Stroke, ensuring bedside deterioration triggers a guideline-concordant, physician-led response.

Accreditation support: Defined escalation triggers, timed pathways (STAT CT, neurosurgery/ICU notification), and mandatory documentation in the stroke flowsheet and escalation record provide the auditable evidence trail required for Comprehensive Stroke Center (CSC) certification (American Stroke Association International Stroke Center Certification Advanced Certification) and AHA Get With The Guidelines®-Stroke performance reporting.

Approval & Sign-Off

Role	Name & Title	Signature	Date
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Approved By (Chief Nursing Officer / Nursing Director)			

Effective date: _____ Next review date: _____